

Abdomen Bloating and Swelling - Male (Child)

Algorithm 1383 | Child (0-17 years) | Gender at birth: Male

Definition

- UAT-only assessment of abdominal bloating or swelling across adult and pediatric patients; not approved for real-patient or production use.
- Child patient.
- Male.

Initial assessment

1. [OPEN_TEXT] What is the patient's age and sex, when did swelling start, and is pregnancy possible or confirmed or has there been a recent delivery or abdominal procedure?
2. [TEMPERATURE] What is the temperature, if measured, and is there severe or localized pain, breathing difficulty, faintness, a rigid abdomen, or rapidly increasing swelling?
3. [YES_NO] Is there green or bloody vomit, vomiting blood, blood or black stool, inability to pass stool/gas/urine, reduced feeding or wet diapers in a child, a lump, weight loss, immunocompromise, or medicines that may cause constipation?
4. [CAREGIVER_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

Triage and redirect questions

1. Redirect → Nausea

Is nausea or vomiting the main concern rather than abdominal bloating or swelling?

Symptoms dominated by nausea or vomiting belong in the Nausea pathway.

1. Disposition level 100

Is there severe sudden or rapidly worsening abdominal pain, a rigid or very tender abdomen, vomiting blood, green vomit in a child, collapse, severe breathing difficulty, heavy bleeding, or pregnancy/postpartum with severe pain or faintness?

NHS.UK bloating guidance lists these as emergency-assessment criteria.

1. Disposition level 70

Is there bloating with persistent vomiting, diarrhea or constipation, pain, fever or chills, a lump, inability to pass urine/stool/gas, blood or black stool, weight loss, reduced child feeding or urine, possible pregnancy, recent surgery, immunocompromise, or a medicine-related concern?

NHS.UK guidance lists these as reasons for urgent clinical review.

1. Disposition level 15

For an otherwise well adult who is not pregnant or recently postpartum, is this mild short-lived bloating with none of the emergency, urgent, pediatric, or vulnerability features above?

NHS.UK guidance describes mild bloating as manageable at home with diet and lifestyle changes.

Advice

Emergency bloating precautions

- Call Qatar 999 and do not self-drive. Do not give food, drink, laxatives, enemas, or pain medicines unless directed by an emergency clinician. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: worsening pain, rigidity, collapse, bleeding, or vomiting; worsening breathing difficulty.

Urgent bloating review

- Arrange same-day in-person assessment through the Qatar route approved for this UAT environment. Do not recommend a laxative, antacid, antiemetic, or other medicine until age, pregnancy, obstruction risk, comorbidity, allergies, and current medicines have been checked.
- Call Back If: pain, vomiting, distension, or fever worsens; blood, black stool, inability to pass stool/gas/urine, faintness, or reduced responsiveness develops.

Home care for mild bloating

- Stay physically active, chew with your mouth closed to avoid swallowing air, drink enough water, eat smaller and more frequent meals, and gently massage the stomach from right to left. Avoid fizzy drinks, excess caffeine and alcohol, gas-producing foods, and large late meals.
- Call Back If: pain, vomiting, or fever develops; does not improve with self-care.

First aid

- Call Qatar 999 and do not self-drive. Do not give food, drink, laxatives, enemas, or pain medicines unless directed by an emergency clinician. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Bloating", <https://www.nhs.uk/conditions/bloating/> (page last reviewed 21 January 2026)

Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from cited public sources; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect. Demographic variants contain only source-supported differences; identical thresholds are documented rather than fabricated. Local destinations and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

Manifest hash: a71746e419115dcd5e27359c03c5dc7b6d0ca0b0af306fc4847b3d4d7ae7a35c | Status: BLOCKED_BY_RESEARCH_GAP